

本文件已電子驗章

國泰綜合醫院

Progress Note

病患姓名:祝鳳英 病歷號碼:0028018818 科別:復健科
 性別:女 病床號:72802 出生日期:1975/10/02
 年齡:50 主治醫師:黃雅鈴 記錄日期:2026/07/2014:00
 文件編號: 00100017202607201403001377

Problem List		
No	Status	Problem

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|---|-------|--|
| 1 | Act | # Anterior communicating artery aneurysm rupture with extensive SAH (subarachnoid hemorrhage) and hydrocephalus s/p (status post) 1. EVD (Extraventricular Drainage) with ICP (intracranial pressure) monitoring insertion (via right Kocher' s point) (Codman ICP (intracranial pressure)) on 2025/03/15 ;
2. Trans-arterial embolization of anterior communicating artery aneurysm (simple coiling) , sonography guided femoral puncture + DSA (Digital Subtraction Angiograph) Bilateral ICA (internal carotid artery) , Left VA |
| 2 | InAct | (vertebral artery) , Right femoral artery on 2026/03/15 , with bilateral hemiplegia , post-stroke spasticity , and cognitive impairment

Hypertension |
| 1 | Act | # Anterior communicating artery aneurysm rupture with extensive SAH (subarachnoid hemorrhage) and hydrocephalus s/p (status post) 1. EVD (Extraventricular Drainage) with ICP (intracranial pressure) monitoring insertion (via right Kocher' s point) (Codman ICP (intracranial pressure)) on 2025/03/15 ; |

S : no somnolence

O : bilateral leg spasticity improved a little from Befon 2# QID
 right leg R2 150'
 left leg R2 135'

right hand touch chin with elbow suspension
 left hand can touch nose without assistance
 faster cognitive response

[Motor Status]

Brunnstrom :

RUE(P/D) : IV/V LE : IV

LUE(P/D) : V/V LE : IV (may be underestimated due to spasticity)

MMT :

RUE(P/D) : 2 /3 , RLE(P/D) : 2-/3-

LUE(P/D) : 3-/3 , LLE(P/D) : 2-/3-

DTR(R/L)

Elbow flexor : +++/+++

Elbow extensor : +++ /+++

Brachialis : +++ /+++

Knee extensor : +++/+++

Ankle plantarflexor : +++/+++ no clonus on exam

Spasticity (MAS) : (R/L)

Elbow flexor : 1/1

Wrist flexor : 0/0

Finger grasping : 0/0

Knee flexor : 3/3

Ankle plantarflexor : 1+/1+

Pathologic Reflexes : (R/L)

Babinski : -/-

Hoffmann : -/-

Gegenhalten (+ , right arm) , bradykinesia(+/- bilateral arms)

A :

Anterior communicating artery aneurysm rupture with extensive SAH

(subarachnoid hemorrhage) and hydrocephalus s/p (status post) 1. EVD (Extraventricular Drainage) with ICP (intracranial pressure) monitoring insertion (via right Kocher' s point) (Codman ICP (intracranial pressure)) on 2025/03/15; 2. Trans-arterial embolization of anterior communicating artery aneurysm (simple coiling), sonography guided femoral puncture + DSA (Digital Subtraction Angiograph) Bilateral ICA (internal carotid artery), Left VA (vertebral artery), Right femoral artery on 2026/03/15, with bilateral hemiplegia, post-stroke spasticity, and cognitive impairment

Hypertension

- P: 1. VPS performed on 7/15
2. Befon 2# QID
3. Performed shockwave therapy to the
- right hamstring (1cm , 2cm) on 7/14
- left hamstring (1cm, 2cm) on 7/16, 7/20
4. Keep rehab programs and encourage bedside stretching exercise

主治醫師之指示或教學內容：

Post-op care

Keep baclofen and may increase dosage if necessary

Encourage stretching exercise

- 2 InAct 2. Trans-arterial embolization of anterior communicating artery aneurysm (simple coiling), sonography guided femoral puncture + DSA (Digital Subtraction Angiograph) Bilateral ICA (internal carotid artery), Left VA (vertebral artery), Right femoral artery on 2026/03/15, with bilateral hemiplegia, post-stroke spasticity, and cognitive impairment

Hypertension

記錄者：吳一宏

記錄醫師：吳一宏

主治醫師：黃雅鈴